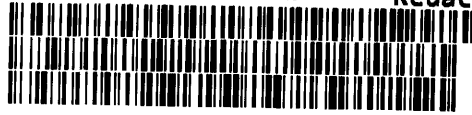


UUID: C8B2F186-9AD3-4F1B-BD3A-7122A0571B96
TCGA-EM-A2CN-01A-PR Redacted



Surgical Pathology Consultation Report (

Patient Name:

MRN:

DOB:

Gender: M

HCN:

Ordering MD:

Copy To:

Service:

Visit #:

Location:

Facility:

Accession #:

Collected:

Received:

Reported:

Specimen(s) Received

1. Nck: left paratracheal node dissection
2. Lymph node: pretracheal lymph node
3. Thyroid: total thyroid

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HIPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Noted		X
Case by (circle):	QUALIFIED	DISQUALIFIED
Reviewed (initials)	Reviewed: 6/18/11	6/17/11

Diagnosis

1. No pathological diagnosis: Lymph nodes, 2 (left paratracheal) dissection specimen
2. No pathological diagnosis: Lymph node, 1 (paratracheal) biopsy
3. Angioinvasive papillary carcinoma, 6.0 cm, follicular variant with focal poorly differentiated carcinoma (less than 5%), right: Thyroid, total thyroidectomy specimen.

Synoptic Data

Procedure:

Received:

Specimen Integrity:

Specimen Size:

Other: Total thyroidectomy with left paratracheal node dissection

Fresh

Intact

Right lobe:

6.5 cm

4.7 cm

4.5 cm

Left lobe:

4.4 cm

2.5 cm

1.8 cm

Isthmus +/- pyramidal lobe:

3.8 cm

1.8 cm

0.8 cm

Additional specimen: Left paratracheal node dissection

1.2 cm

0.9 cm

0.3 cm

Specimen Weight:

Tumor Focality:

Unifocal

----- DOMINANT TUMOR -----

Tumor Laterality:

Right lobe

1CΔ-0-3

carcinoma, papillary, follicular variant

8340/3

Site: thyroid, NOS C73.9

hw 6/18/11

Tumor Size:	Greatest dimension: 6.0cm Additional dimension: 4.5 cm Additional dimension: 4.0 cm
Histologic Type:	<i>for TSS, follicular variant = 100%</i> Papillary carcinoma, follicular variant Papillary carcinoma, other variant: Focal poor differentiation* (less than 5% of tumor) Follicular architecture Classical cytomorphology
Histologic Grade:	Not applicable
Margins:	Margins uninvolved by carcinoma Distance of invasive carcinoma to closest margin: 0.01mm
Tumor Capsule:	Partially encapsulated
Tumor Capsular Invasion:	Present, extent widely invasive
Lymph-Vascular Invasion:	Present, extensive extent (4 or more vessels)
Perineural Invasion:	Not identified
Extrathyroidal Extension:	Not identified
TNM Descriptors:	Not applicable
Primary Tumor (pT):	pT3: Tumor more than 4 cm, limited to thyroid or with minimal extrathyroidal extension (eg, extension to sternothyroid muscle or perithyroid soft tissues)
Regional Lymph Nodes (pN):	pN0: No regional lymph node metastasis Number of regional lymph nodes examined: 3 Number of regional lymph nodes involved: 0
Distant Metastasis (pM):	pM1: Distant metastasis Site(s) of metastasis: C6 soft tissue Source of pathologic metastatic specimen

*Pathologic Staging is based on AJCC/UICC TNM, 7th Edition

Electronically verified by:

Clinical History

thyroid ca

Gross Description

1. The specimen container labeled with the patient's name and as "Left Paratracheal Node Dissection", contains two pieces of fibrofatty tissue measuring 0.8 x 0.5 x 0.3 and 1.2 x 0.9 x 0.2 cm. received in 10% buffered formalin.

1A submitted in toto.

2. The specimen container labeled with the patient's name and as "Paratracheal Lymph Node" contains one piece of red tan tissue that measures 0.5 x 0.3 x 0.2 cm. received in 10% buffered formalin.

2A submitted in toto.

3. The specimen labeled with the patient's name and as "Total Thyroid [Stitch = RT Superior Pole]" consists of a thyroid gland that is oriented with a suture and weighs 69.7 g. The right lobe measures 6.5 cm SI x 4.7 cm ML x 4.5 cm AP, the left lobe measures 4.4 cm SI x 2.5 cm ML x 1.8 cm AP and the isthmus measures 3.8 cm SI x 1.8 cm ML x 0.8 cm AP. The external surfaces have fibrous adhesions. No parathyroid glands and/or no lymph nodes are identified grossly. The external surface is painted with Silver nitrate. The entire right lobe is replaced by a well delineated solid nodule. The remainder of the thyroid tissue is grossly unremarkable. Sections of the nodule and normal tissue are stored frozen.

Representative sections of the specimen are submitted as follows:

3A-M right lobe, superior to inferior (B-E F-I sections quadrisectioned)

3N-O isthmus (submitted in toto)

3P-T left lobe, superior to inferior (submitted in toto)